

REPRODUCTIVE HEALTH SUB-WORKING GROUP RHSWG

NEWSLETTER ISSUE #6

APR-JUN 2025

In Lebanon, the Reproductive Health Sub-Working Group (RHSWG) was established in 2013 under the Health Sector. The Ministry of Public Health (MOPH) leads the RHSWG where the United Nations Population Fund (UNFPA) serves as the co-lead. The RHSWG members include national and international non-governmental organizations (NGOs), academics, relevant ministries, and UN agencies who coordinate to standardize and improve all aspects of sexual and reproductive health (SRH) programming such as community awareness, service quality, accessibility, and coverage. The RHSWG develops and follows up on the implementation of a yearly action plan that contributes to the objectives laid out in the Lebanese Response Plan (LRP).

This issue highlights priority SRH needs identified and issues faced between April and June 2025

✓ RHSWG Key Achievements Apr-Jun 2025 include:



Maternal and Neonatal Secondary Healthcare (Advocacy)

Following UNHCR's announcement in the last week of May on their plans to end their referral care support program on Nov 30, 2025. The RHSWG released an advocacy document on the [implications of the cessation of UNHCR's referral care support](#) for vulnerable women and newborns in Lebanon.

The brief highlights concerns about increased risk in maternal and neonatal mortality following UNHCR's exit due to financial barriers to accessing hospital deliveries and neonatal intensive care unit (NICU) services. It ends with a call for action to mobilize further resources and leverage existing community midwifery projects to ensure all vulnerable women access skilled healthcare professionals during delivery



Enhancing Referrals

In an effort to identify geographical gaps and improve referrals between health partners, the RHSWG completed two mapping exercises for projects supporting Community Midwifery interventions and financial support for Neonatal and Pediatric Intensive Care Unit services (NICU/PICU) at hospitals.

- ➡ [NICU/PICU Support Mapping and Referral Matrix - July 2025 \(Snapshot\)](#)
- ➡ [Community Midwifery Mapping & Referral Matrix - May 2025 \(Snapshot\)](#)



Dashboard Updates

- ➡ The RHSWG released the [NICU Hospital Support Dashboard](#) to reflect the status of hospitals providing subsidized Neonatal Intensive Care Unit (NICU) services and improve referrals.
- ➡ The RHSWG also updated the [Support for Deliveries at Hospital Dashboard \(June 10, 2025\)](#) to reflect the status of hospitals providing subsidized deliveries and identify gaps in service provision due to hostilities.

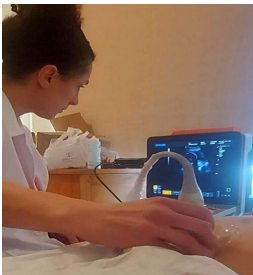




NEWS ALERT!

Due to the significant reduction in funding, UNHCR has been forced to take the difficult decision of phasing out its secondary health care programme in Lebanon by the end of November 2025. After this date, refugees will no longer be covered by UNHCR including acute emergency, delivery, and NICU/PICU conditions. This development will have serious implications and underscores the urgent need to advocate with donors and partners to support access for essential hospital care after November 2025.

*Based on historical data from UNHCR in 2024, **the RHSWG estimates a gap in direct medical cost of \$6M for delivery support and \$4.5M for NICU support.** Without UNHCR support, **approx. 28,000 vulnerable women and 3000 neonates will face financial barriers to accessing safe hospital deliveries & NICU services** increasing the risk of both maternal and neonatal mortality.*



To prevent avoidable maternal and neonatal mortality and morbidity, collaborative efforts are needed to mobilize resources for subsidized institutional deliveries and leverage existing community midwifery projects to ensure all vulnerable women access skilled healthcare professionals during delivery. The RHSWG continues to work with the Health Sector to determine the funds needed to target the most vulnerable women as of 2026 based on the updated PIN, standardized vulnerability criteria, and capacities available.



PUBLICATIONS

UNFPA Launches State of World Population 2025 Report, "The Real Fertility Crisis"

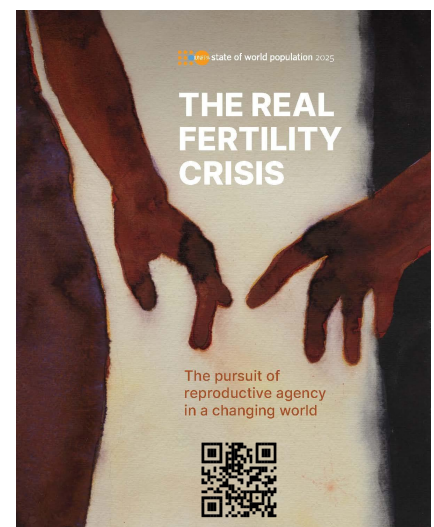
UNFPA has released its 2025 State of World Population (SWOP) report, titled **"The Real Fertility Crisis: The pursuit of reproductive agency in a changing world"**. The report reframes the global discourse on population, asserting that the true fertility crisis is not overpopulation or declining birth rates, but rather the widespread inability of individuals to achieve their own fertility goals. To mark World Population Day, UNFPA Lebanon launched on July 11 this flagship report, with the presence of the Minister of Social Affairs, highlighting that shrinking families are a silent crisis stemming from a lack of genuine reproductive choice rather than a desire for fewer children.

UNFPA asked people – **across 14 countries, which together represent more than a third of the global population** – what they actually want for their reproductive lives and futures, and whether they believe they will be able to realize those ambitions.

The report reveals that a significant number of women and men are unable to realize their reproductive aspirations. The primary barriers identified are systemic, including economic instability, gender discrimination, lack of support from partners and communities, and inadequate access to quality sexual and reproductive healthcare and services like affordable childcare.



Launch of the SWOP 2025 report at LAU, Beirut - July 11, 2025
 Left to Right: UNFPA Regional Director for Arab States Laila Baker, Lebanese Minister of Social Affairs Hanine El Sayed, Officer in Charge UN Resident/Humanitarian Coordinator Mathew Hollingworth, UNFPA Lebanon Representative Anandita Philipose





PUBLICATIONS - cont.

Key stats on the identified barriers to choice are:

-  **20%** of reproductive-age adults believe they will be **unable to have the number of children they desire**
-  **1 in 3** have unintended pregnancies
-  **39%** reported that **financial limitations** had affected or would affect their ability to realize their desired family size
-  **1 in 5** said **fears about the future**, such as climate change, environmental degradation, wars and pandemics, would lead or had led to them having fewer children than desired
-  **1 in 4** have felt **unable to fulfil their desire for a child** at their preferred time.



In this [video](#), UNFPA Lebanon asked people across generations about their fertility choices. Check out what they say!

The report calls to foster "reproductive agency"— a person's ability to make free and informed choices about sex, contraception, and starting a family. This requires a policy shift away from demographic targets and toward creating an enabling environment that empowers individuals to realize their fundamental reproductive rights and choices



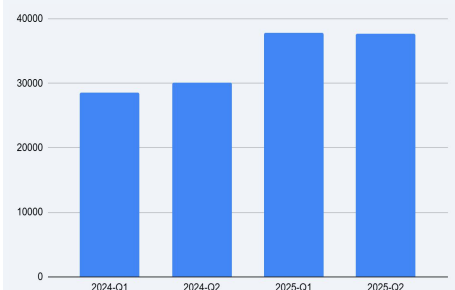
PRIMARY HEALTHCARE - Service Utilization Trends

The health sector in Lebanon continues to deliver equitable quality healthcare services targeting vulnerable Lebanese and non-Lebanese individuals including displaced Syrians, Palestinian Refugees from Syria (PRS), Palestine Refugees in Lebanon (PRL), migrants, and displaced individuals of diverse nationalities. The health sector supports improved access to comprehensive primary healthcare which includes sexual and reproductive healthcare among other essential health and nutrition services.

Analysis: As of Q2 2025, there is a 28% increase in the number of subsidized Antenatal Care (ANC) consultations at Primary Healthcare Centers (PHCCs) across Lebanon compared with the same period in 2024. However, there was a notable decrease in the month of June 2025 compared to May 2025. Main reasons include religious holidays in Q2 (Adha/Ashoura) leading to facility closures and people deprioritizing healthcare visits. Further challenges for June included the decrease in the number of people coming to the PHCC given the security situation.

Subsidized ANC Consultations

Source: Activity Info



Challenges reported by health partners include the unavailability of Midwives in remote areas in Akkar governorate and unavailability of Midwives and OBGYNs in border areas in Nabatieh/South governorates with many still not returned post-ceasefire.

Further support is needed to enhance SRH service provision across the country especially with some partners closing out projects as of Q3 2024. Newcomers from Syria in Akkar and Baalbak El Hermel continue to live in poor WaSH conditions at informal collective shelters which put women and girls at increased risk of UTIs and reproductive health tract infections. Health partners have also reported barriers to access that include legal status, transportation costs, and lack of awareness of available services, **most importantly subsidized deliveries at hospitals.**

ACCESS TO MATERNAL CARE AT THE SECONDARY LEVEL - Outlook into Q3 & Q4 2025

In Q2 2025, health partners continued to **subsidize deliveries** at 40 supported hospitals. Looking into the second half of 2025, significant funding cuts are expected! Notably UNHCR's phasing out of its referral care program (Q4 2025), and the conclusion of the LHF 1st allocation and other emergency funds (Q3 2025). Accordingly, collaborative efforts are needed to mobilize resources for subsidized institutional deliveries (**especially high risk pregnancy and the most vulnerable pregnant women**) and leverage existing community midwifery projects to ensure all vulnerable women have access to family planning, antenatal care, safe delivery, and postnatal care services to prevent avoidable maternal and neonatal mortality and morbidity.

SEXUAL VIOLENCE & CLINICAL MANAGEMENT OF RAPE SERVICES

Globally, **1 in 3 women are subjected to physical or sexual violence by an intimate partner or a non-partner** (WHO).

The **Gender-Based Violence Information Management System (GBV IMS)** reports a similar percentage for Rape and Sexual Assault incidents when compared to Q1 2025 (3% rape and 14% sexual assault).

The **MOPH PHENICS** reports **double the number of beneficiaries of CMR services when compared to Q1 2025**.

- ➔ 4/14 of MOPH CMR facilities have reported on cases for Q2 2025. The facilities are included in Nabatieh, Baalbek, Beirut, and Tripoli districts.
- ➔ There was zero reporting for the month of April.
- ➔ Key statistics include:
 -  Approx. **64%** (43% in 2024) of reported survivors receiving CMR services at CMR clinics **are minors**
 -  Approx. **71%** of reported survivors receiving CMR services at CMR clinics **are female**
 -  More than **72%** (57% in 2024) of reported survivors sought services at CMR facilities **after 72 hours** of the rape incident

All 14 CMR facilities (8 hospitals and 6 PHCCs) continue to be supported by health partners to offer free-of-charge CMR services to rape survivors of all nationalities.

CMR Capacity Building Trainings under UNFPA

There continues to be a need to expand awareness of CMR services where the CMR TF continues to work with MOPH to enhance integration of CMR in awareness sessions at the community and PHCC levels. A training initiative that aimed to improve the response to sexual and gender-based violence (SGBV) by providing specialized Clinical Management of Rape (CMR) sensitization training to **over 1,200 professionals in Lebanon**. The program is designed to create a coordinated and survivor-centered system of care.

The training targets four key groups:

-  **182** Mental health professionals
-  **160** GBV actors
-  **100** Hotline operators
-  **812** staff of governmental hospitals



"The training didn't just equip me individually; it contributed to a stronger, more informed network of psychologists. We are now better prepared, collectively, to offer critical, holistic support to every survivor who reaches out" - clinical psychologist

This [video](#) raises awareness on sexual assault and provides critical information on CMR service provision

United Nations Population Fund (UNFPA)

Donors: KOICA, ECHO, SIDA, CERF, France, OCHA, UNFPA Emergency Fund

UNFPA coordinated closely with the MOPH, the Ministry of Social Affairs (MoSA), and local partners to adapt its response strategy to meet the emerging needs of the affected population and ensure continuity of essential SRH services at community, primary care and secondary care levels..

SERVICE DELIVERY



UNFPA's **support to 21 hospitals** across Lebanon, included financial coverage for childbirth costs which enhanced access to institutional based **safe delivery for 550 pregnant women**. UNFPA support included access to emergency obstetric care for pregnant women experiencing complications.



UNFPA's support to MOPH's network of **CMR facilities** through procurement and prepositioning of essential medical supplies and medication allows survivors of rape **access to lifesaving medical services**. UNFPA' support also included financial coverage for CMR services at 3 CMR facilities.



Between April and June 2025, UNFPA supported access to SRH services and information to a total of **34,946 people** in partnership with SIDC, Akkarouna, Caritas, Salama, Amel, Makassed, SWSL, LOM and Marsa in **53 PHCCs and medical mobile units** that operate in various communities across Lebanon. These services targeted vulnerable groups, including Syrian refugees, internally displaced persons, people with disabilities, and the LGBTQI community. Support varied between comprehensive SRH services and Information.

VIRTUAL TOURS

Learn more about UNFPA service delivery activities by meeting health practitioners & social workers in two new virtual tours funded by **KOICA**



SRH services and Information: Included pre-and postnatal care, menstrual management, sexually transmitted infections, family planning, and nutrition for pregnant and breastfeeding women etc UNFPA supported services includes medical consultation, midwifery care, subsidization of laboratory tests, sonography and RH tests, outreach awareness raising activities in addition to supporting referrals to specialized services as needed (i.e mental health care etc).

Psychosocial support (PSS) and awareness on SRH: At the request of MOPH and in collaboration with the Social Worker Syndicate in Lebanon, UNFPA deployed social workers to provide PSS and awareness on SRH at 15 PHCCs across Lebanon reaching 2515 beneficiaries

Midwifery care and information on SRH: In partnership with LOM, UNFPA deployed midwives to provide midwifery care and awareness on SRH at 10 PHCCs across Lebanon. reaching 5509 beneficiaries with midwifery care services and information

UNFPA - cont.

CAPACITY BUILDING TRAININGS

A training program for **259 healthcare providers on obstetric emergencies**, was conducted across **17 government hospitals**. The goal was to enhance the skills of doctors, nurses, and midwives in managing life-threatening situations during childbirth, such as postpartum hemorrhage, eclampsia, and obstructed labor. By equipping a significant number of providers with the necessary knowledge and practical skills, the program aims to reduce maternal and neonatal morbidity and mortality rates.

UNICEF

Donors: Korea, BPRM, Australia

NICU-PICU Support - UNICEF is supporting through Korea's grant 11 hospitals with NICU and PICU cases. During the months of April-June 2025, 327 neonates and pediatrics were supported, with out-of-pocket payments, for them to access lifesaving services in NICU and PICU out of which 186 were males and 141 were females.

Hayat Newborn Kit - 227 NICU patients in 11 hospitals received the Hayat Newborn Kit during Q2. The kit contains items to support the mother's postpartum journey and the care for the baby. Items include: thermometer, scratch mittens, short sleeved and long-sleeved vests, clothes, condoms and diapers.

Procurement - Under BPRM and Australia, UNICEF procured NICU equipment and supplies to support 11 hospitals. These equipment include: Hayat New born Baby Boxes and BCPAP consumables.

International Medical Corps (IMC) - Lebanon Humanitarian Fund (LHF)

A New Life, A New Beginning for Ghoson and Her Baby

Saida, Lebanon — In the early morning, 21-year-old Ghoson, a displaced Syrian woman in her first pregnancy, arrived at Saida Governmental Hospital in distress. Past her due date, with no documents and fearful for her baby's safety, she faced an emergency situation. A fetal heart monitor revealed alarming signs: her baby was in distress, and an immediate C-section was required.

Thanks to support from IMC through an LHF-funded project, Ghoson was admitted and received full medical care — her only chance for a safe delivery.

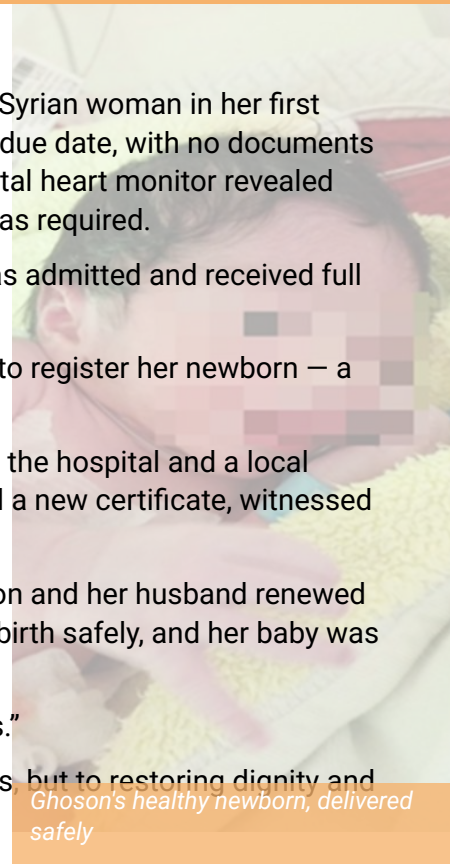
A Legal Challenge: Without a marriage certificate, she wouldn't be able to register her newborn — a barrier with long-term legal implications.

A Unique Solution: Recognizing the urgency, IMC staff coordinated with the hospital and a local marriage officer. Moved by her story, he came to the hospital and issued a new certificate, witnessed by the medical team.

Hope Restored: What began as a crisis became a moment of joy: Ghoson and her husband renewed their vows at the hospital, surrounded by smiles and support. She gave birth safely, and her baby was registered with dignity.

"For a brief, beautiful moment, love and life triumphed over war and loss."

This story is a testament to IMC's commitment — not only to saving lives but to restoring dignity and hope. Ghoson's healthy newborn, delivered safely



Premiere Urgence Internationale (PUI)

European Civil Protection and Humanitarian Aid Operations (ECHO)

Meeting Urgent Maternal Health Needs Amid Displacement

At 23, Zainab, a Syrian woman displaced in Lebanon in 2024, faced significant hardship in her first pregnancy due to lack of income and inadequate access to healthcare and adequate nutrition.

During a community outreach visit, PUI midwife identified Zainab as severely malnourished, with a MUAC of just 18 cm, anemic, and suffering from a severe urinary tract infection and vaginitis – all while in the late stages of her first pregnancy. PUI urgently referred Zainab to a specialized malnutrition treatment center, where she began receiving essential nutritional support and regular medical monitoring.

After enrolling in the program, Zeinab completed three antenatal care visits in Serepta PHCC and received continuous support from PUI, ensuring the health and well-being of both her and her baby. With time Zainab gradually improved and her pregnancy progressed smoothly, with both Zainab and her baby showing signs of good health.

On May 28, 2025, Zainab safely delivered a healthy baby boy via C-section at Nabatieh Governmental Hospital. She attended her first postnatal care visit on 5 June 2025 and is now successfully breastfeeding. Both mother and newborn are in good health.

"You gave me a chance when I was at my lowest. I am truly blessed. Thank you for giving me strength and support every step of the way," said Zainab.

Zainab's story powerfully shows how humanitarian support transforms lives. Without the timely intervention, her path could have taken a more difficult turn. Thanks to funding from EU Humanitarian Aid, women like Zainab are receiving the vital care and assistance they deserve.



A midwife conducting a postnatal assessment for Zainab during a PNC session at her home, South Lebanon, June 2025

Action Against Hunger (ACF) - Lebanon Humanitarian Fund (LHF)

ECHO Fund

Improving Maternal Health: Insights from the ACF HRP Patient Exit Survey

ACF's LHF-funded program supported 255 high-risk pregnancies at four governmental hospitals (Baalbeck, Hermel, Saida, Nabatiyeh), offering antenatal, delivery, and postnatal care.

Between January and April 2025, ACF conducted a patient exit survey to evaluate the hospitalization coverage program for high-risk pregnancies (HRP). ACF staff systematically contacted all discharged patients to gather feedback on their experiences and the quality of care received.

Key Findings:

- **Access and Satisfaction:** 54% rated antenatal care as excellent, 41% as good; 97% were satisfied with hospitalization and delivery services.
- **People-Centered Care:** 84% reported clear explanations from healthcare staff; 57% rated facility cleanliness as excellent; 86% felt respected; 84% felt supported.
- **Barriers:** Financial challenges were significant; 94% reported no additional payment requests; 96% faced no cultural barriers.
- **Feedback:** 67% felt confident in lodging complaints.

To further improve maternal health outcomes, the following recommendations were made:

- Ensure sustained funding for maternal health interventions.
- Expand financial assistance to include neonatal intensive care services.
- Upgrade health facility infrastructure to improve hygiene and patient comfort.
- Strengthen community outreach and health education initiatives.
- Enhance integrated maternal and child health services at the community and PHCC level.
- Invest in capacity building for hospital staff, focusing on both physical and emotional support.
- Continue monitoring patient feedback to inform and improve maternal and newborn health interventions.

Conclusion

The HRP Patient Exit Survey underscores the importance of equitable, high-quality healthcare services for pregnant women in Lebanon. By addressing the identified barriers and implementing the recommended actions, we can support better health outcomes for mothers and their infants, ultimately contributing to a healthier and more resilient community.

Relief International (RI) - Lebanon Humanitarian Fund (LHF)

A Journey of Strength: A Mother's Story from Arsal

A 35-year-old Lebanese mother of two from Arsal, with a background in nursing and over a decade of experience at a PHCC, found herself facing mounting financial responsibilities. Her husband, who works as a volunteer with civil defense and a wedding photographer, doesn't have a fixed job. To support her household, she continued working as a nurse despite the challenges. During her third pregnancy, in the first trimester, unexpected complications emerged. While attending antenatal care at Al Iman PHCC, she suffered a stroke (DVT), and her blood pressure began to rise. Her gynecologist warned her that she would need to deliver in a hospital due to

Relief International (RI) - Cont.

the risks involved. At this point, overwhelmed by anxiety and financial fears—thinking about surgery, tests, ICU care, and hospital admission without any medical insurance—she was introduced to Relief International. This moment marked a turning point.

“It really made the difference,” she shared.

Through their healthcare program for high-risk pregnancies, Relief International stepped in, covering the costs that her family could not manage. She received consistent medical follow-up and treatment. Under the MOPH coverage and Relief International contribution, she was admitted to Al Rayan Hospital in Baalbek, where she underwent a successful C-section. She gave birth to a healthy baby and received excellent care during her stay. She was discharged in good health, returning home safely with her newborn and husband.

Two weeks later, she visited the PHCC for her postnatal care appointment, accompanied by her new baby for a routine follow-up.

“The support I received from Relief International came at the right time and made a significant difference,” she repeated, highlighting the deep impact of the intervention.



Post natal care visit at Iman PHCC - Aarsal, where the mother received IYCF counseling from a Lactation Specialist deployed by Relief International (RI).

GET INVOLVED



To receive this newsletter, RHSWG announcements, and to join the working group [please sign up here](#).

The [RHSWG Google Drive Folder](#) contains repositories of SRH guidance documents, RHSWG and CMR TF MoM, communication materials, service mapping, and other useful documents.

All 35 organizations mentioned are active members of the RHSWG: ACF, Al Makassed, AMEL, BASD, Caritas Lebanon, Gawth, Humedica, IMC, IOCC Lebanon, IOM, IRC, ICRC, Lebanese Order of Midwives (LOM), La Chaîne De l'Espoir, LFPA, LRC, Mercy USA, MDM, MARSA, MEDAIR, MoPH, Makhzoumi, Order of Malta, Oxfam, Plan International, Proud Lebanon, PU-AMI, RI, SIDC, SALAMA, UNFPA, UNHCR, UNICEF, UNRWA, and WHO.

According to RHSWG SRH Service Mapping Exercise conducted in Q2 2025, the major donors supporting SRH interventions (by alphabetical order): AFD (France), BHA-USAID and BPRM (United States of America), BMZ-DHK (Germany), CERF, CDCS (The Crisis and Support Centre), EU-NDICI and ECHO (European Union), GAC (Canada), GFFO (Germany), Global Fund, IPPF, LHF, Monaco-Foundation Merieux, NORWAC, Private Funds, Republic of Korea, Swedish International Development Agency (SIDA)

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